

“Who Hasn’t Given Birth?”

Counter–Narratives of Childbirth Injury on Chinese Social Media

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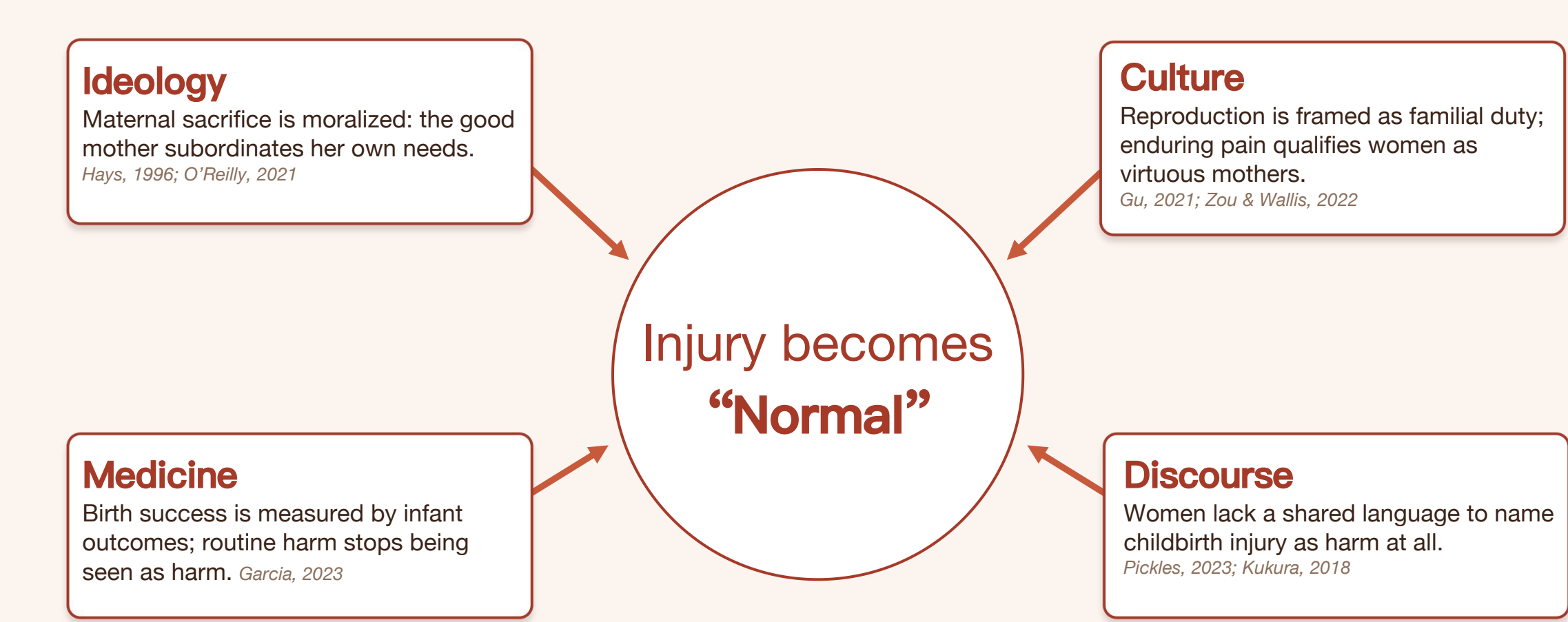
1 Why This Study

35% of women live with chronic health conditions after childbirth, at least **40 million** each year (Vogel et al., 2024). Yet childbirth injury remains largely unspeakable, absorbed into expectations of maternal endurance.

In China, three-child policy discourse frames reproduction as national contribution while saying little about its bodily costs.

How do women narratively resist the normalization of childbirth injury?

2 How Injury Becomes “Normal”



3 Research Questions

RQ1 • What counter-narrative themes emerge, and what normalizing discourses do they contest?

RQ2 • What narrative strategies do users employ?

RQ3 • How do users position themselves against dominant motherhood narratives?

4 Framework

Counter-narrative theory: stories that resist dominant cultural narratives (Andrews, 2004).

Positioning: narrating against a master script is an act of self-positioning (Bamberg & Wipff, 2021).

Counterstory and narrative repair (Nelson, 2001).

5 Data & Method

Site • Douban: a group (Alliance of Childbirth Experiencers) + open topic threads

Data • ~35 posts with comment threads + 3 topic threads, 485+ contributions, ~192,000 Chinese characters (Feb 2021 - Mar 2026)

Analysis • Phase 1: thematic analysis (Braun & Clarke, 2006) → RQ1. Phase 2: narrative and positioning analysis → RQ2, RQ3

6 Findings

1 • Naming Bodily Harm

testimonial specificity → counters discursive unspeakability

“I just coughed and leaked urine again. I truly feel I have no dignity as a human being.”

2 • The Unseen Mother

contrastive witness → counters the child-centered relational order

“Who hasn’t given birth? Why are you so delicate?”

3 • The Unequal Cost

embodied accounting → counters the ideology of worthwhile sacrifice

“These are non-mandatory options in life. Not experiencing them is just fine.”

4 • Breaking the Chain of Silence

epistemic repair → counters intergenerational knowledge interruption

“What is truly frightening is the opacity of information about this matter.”

7 Contributions

Embodied experience works as a positioning resource: “I was there, this is what happened to my body” grounds every positioning move.

Epistemic repair is a distinct counter-narrative practice: restoring the conditions of speakability, not just contesting claims.

These narratives sit between everyday maternal expression (Das, 2017) and organized feminist activism (Hou, 2020): the line between personal testimony and political resistance is thinner than assumed.

8 Implications

Maternal health messaging fails if it reproduces the logic it targets: framing suffering as a cost worth bearing, or measuring birth success by infant outcomes alone.

The problem is not missing information, but missing conditions for that information to be spoken, heard, and taken seriously.

A cesarean is not simply one cut. It cuts through layer after layer, from skin to fat to fascia to muscle to the uterus.

My friends asked, “How are you?” His relatives headed straight for the crib: “Where’s the baby?”

He seems to have changed in nothing, yet he obtained the same result that I had to go through all of this to get.

Not as sacrifice, not as a medal, but simply as the truth of what a person went through.